

METRO GENERAL HOSPITAL

LEVEL 1 TRAUMA

ERADMISSIONSUMMARY | DOCTOR:K.MILLER,MD

Patient: Doe, John | **Admission:** 02/14/2026 15:42

PRIMARY ASSESSMENT & TRIAGE

Presentation: Pt arrived via EMS following extrication. Noted 12cm stellate laceration to

left cheek/jaw with embedded glass fragments. Pt complaining of radiating spinal pain.

DIAGNOSTICS ORDERED / RESULTS

- **CT Head/Neck:** Negative for intracranial bleed. Positive for cervical disc displacement at C5-C6.
- **X-Ray Lumbar:** Loss of lordosis, suggestive of severe muscular spasm vs structural injury.
- **Lab Work:** Elevated WBC (Stress response). BAC: 0.00.

INTERVENTIONS

Wound irrigated; 42 sutures applied (see surgical log). Pain managed via IV Morphine. Pt stabilized and discharged with "Strict Bed Rest" orders for 14 days and referral to Ortho/Neuro specialists.



METRO GENERAL HOSPITAL - SURGICAL SERVICES

TRAUMACAREUNIT | WOUNDMAPPING&REPAIRLOG

PATIENT: DOE, JOHN

ID: JD-021426

DATE: 02/14/2026

I. CLINICAL PRESENTATION (PRE-OPERATIVE)

Patient arrived via EMS trauma transport. Examination reveals an **extensive 12cm stellate (star-shaped) laceration** located on the left malar region extending to the mandibular border. Wound edges are jagged and irregular, consistent with high-velocity tempered glass intrusion from the B-pillar side-window shattering.

II. FOREIGN BODY REMOVAL

Visual and manual exploration of the wound bed revealed multiple foreign bodies. Three (3) shards of tempered automotive glass, ranging from 2mm to 5mm, were recovered from the subcutaneous adipose layer. Significant debridement of contaminated tissue was required to ensure wound viability.

III. MULTI-LAYER CLOSURE TECHNIQUE

Due to the depth and irregularity of the wound, a single-layer closure was deemed insufficient. A **Multi-Layer Surgical Repair** was initiated:

- **Subcutaneous Layer:** 12 Interrupted Vicryl 4-0 sutures utilized for deep tissue apposition and tension reduction.
- **Epidermal Layer:** 30 Prolene 6-0 sutures utilized for fine skin-edge alignment.
- **Total Suture Count:** 42 sutures.

IMPRESSION: High risk for hypertrophic scarring and permanent cosmetic disfigurement. Wound architecture suggests significant tension across the malar fold, likely resulting in a permanent visible deficit.

Electronically Verified by K. Miller, MD | Chief of Trauma Surgery



CHEMISTRY & HEMATOLOGY PANEL (Stat)

Test Component	Result	Reference Range	Flag
White Blood Cell (WBC)	14.8 x10 ³ /uL	4.5 - 11.0	HIGH (Leukocytosis)
Hemoglobin (Hgb)	13.2 g/dL	13.5 - 17.5	LOW (Mild
Glucose, Random	162 mg/dL	70 - 125	Hemorrhage) HIGH
Blood Alcohol (ETOH)	< 10 mg/dL (Neg)	0.00	(Stress Induced)
Drug Screen (Urine)	Negative	Negative	NORMAL

DIAGNOSTIC RADIOLOGY (ER PRELIMINARY)

Cervical Spine 3-View: Loss of normal cervical lordosis. This is likely secondary to acute muscular guarding and paravertebral spasm. No fracture noted. Recommended CT/MRI correlation for radicular symptoms.

Chest X-Ray (AP): Clear. No pneumothorax. Mediastinal contours are normal despite high-impact B-pillar intrusion.

PATHOLOGY NOTE: Facial wound debridement yielded multiple fragments of tempered glass (approx. 2mm to 5mm). Samples discarded after forensic documentation.



OPERATIVE REPORT

SURGEON: Sarah Spine, MD

ANESTHESIOLOGIST: Victor Vapor, MD (Independent Group #44)

PROCEDURE DATE: May 12, 2026

PREOPERATIVE DIAGNOSIS:

L4-L5 Disc Herniation with Left-Sided Radiculopathy (M54.16).

PROCEDURE PERFORMED:

1. Lumbar Microdiscectomy, L4-L5 Left.
2. Use of Operating Microscope for Microsurgical Dissection.

OPERATIVE SUMMARY:

After induction of general anesthesia, the patient was placed in the prone position on the Andrews frame. A 3cm midline incision was made over the L4-L5 interspace. Using the operating microscope, a laminotomy was performed. The thecal sac was retracted medially to reveal a **large, extruded disc fragment** impinging heavily on the L5 nerve root. The fragment was removed in one piece. The nerve root was then noted to be free and decompressed. Hemostasis was achieved. Closure was performed in multiple layers.

ESTIMATED BLOOD LOSS:

Minimal (< 50cc). No complications encountered.

